



The Faculty of
Intensive Care Medicine



UNIVERSITY OF
BIRMINGHAM

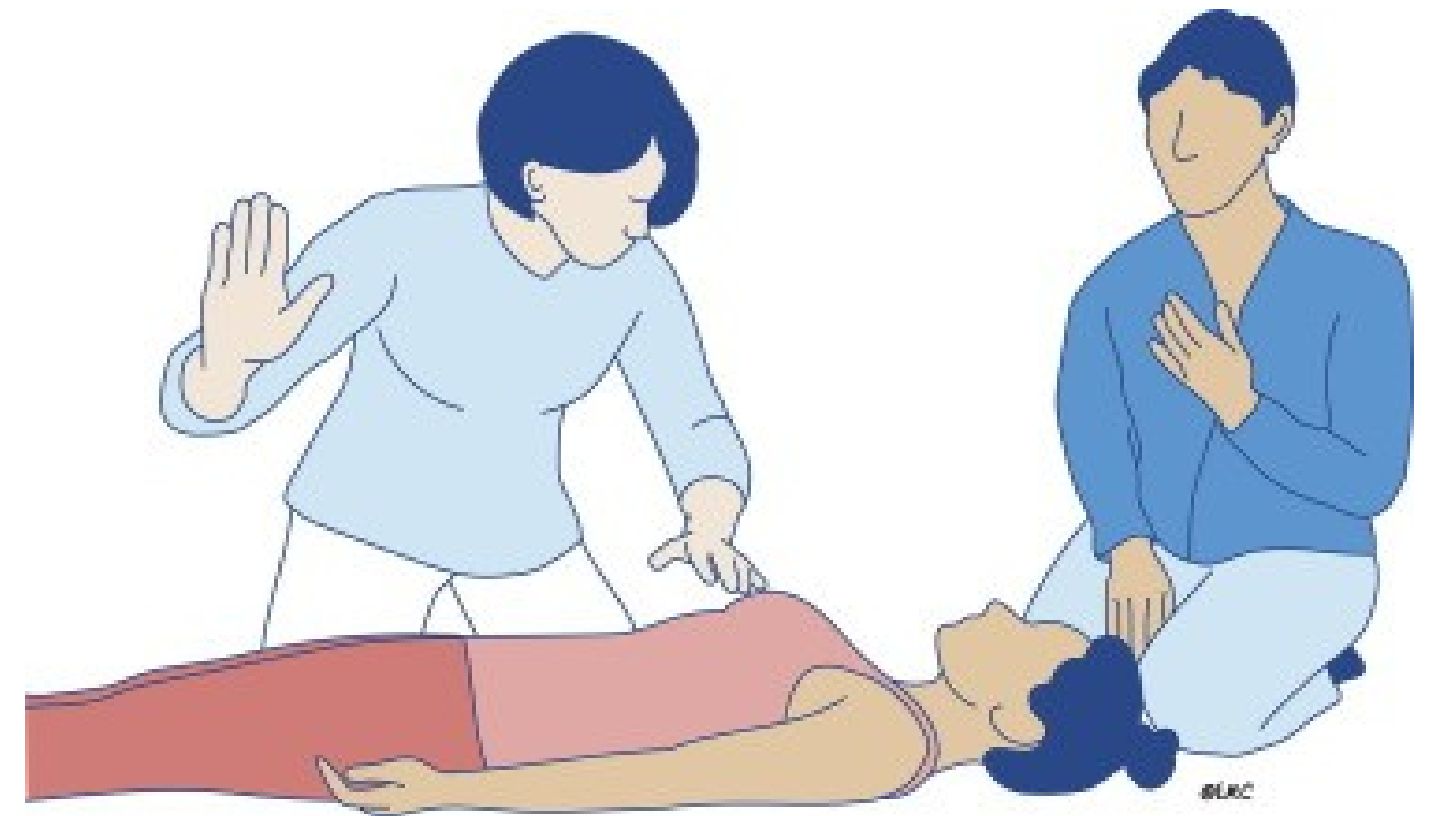
CPR Modifications & First Aid

OBJECTIVES

- ▶ Understand how to manage a more extended range of patients and situations
 - ▶ Unconscious
 - ▶ Infection risk
 - ▶ Children
 - ▶ Drowning
 - ▶ Choking
 - ▶ Bleeding

UNCONSCIOUSNESS

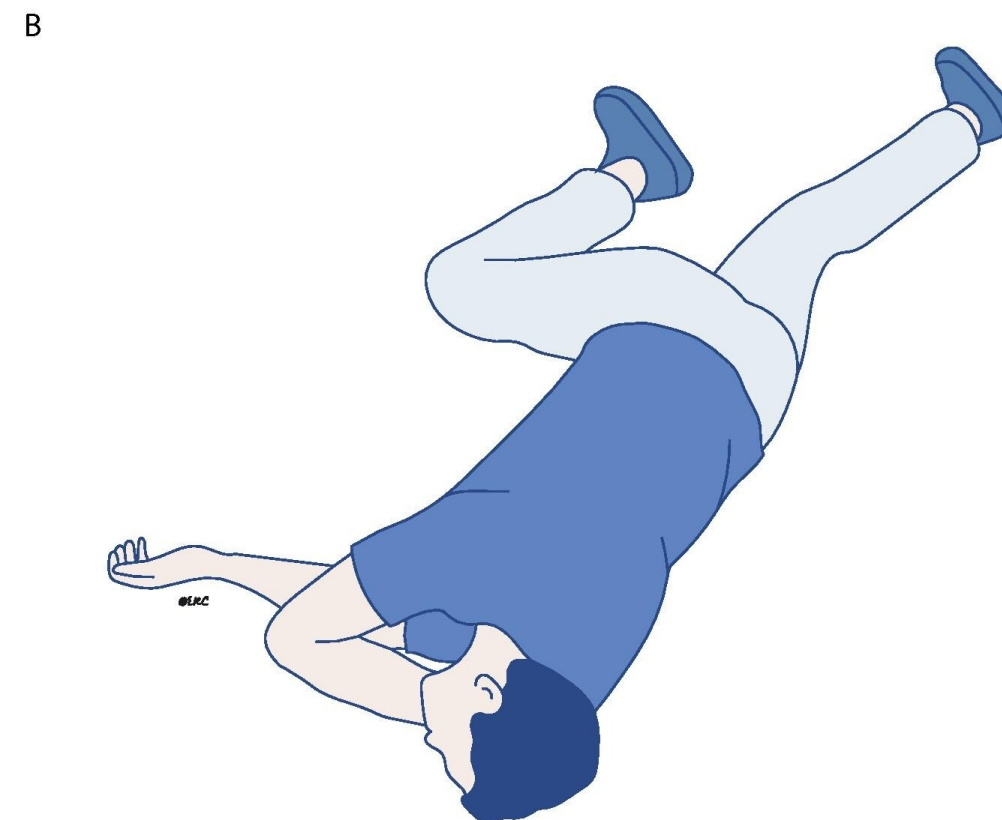
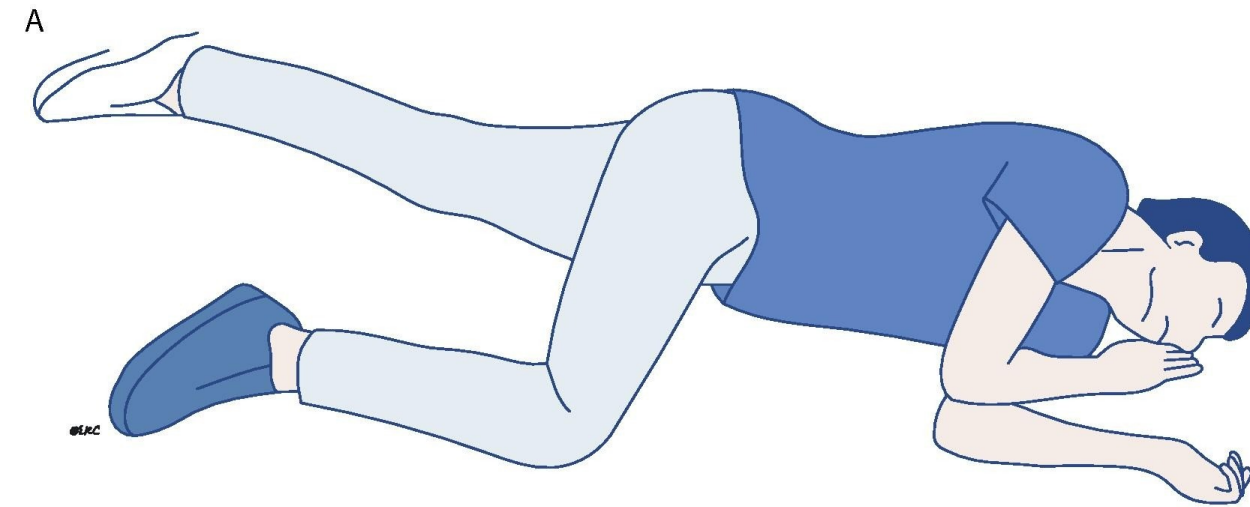
- ▶ Assess the collapsed or unwell person
- ▶ They respond
 - ▶ Leave in the same position
 - ▶ Find out what is wrong
 - ▶ Reassess regularly



UNCONSCIOUSNESS

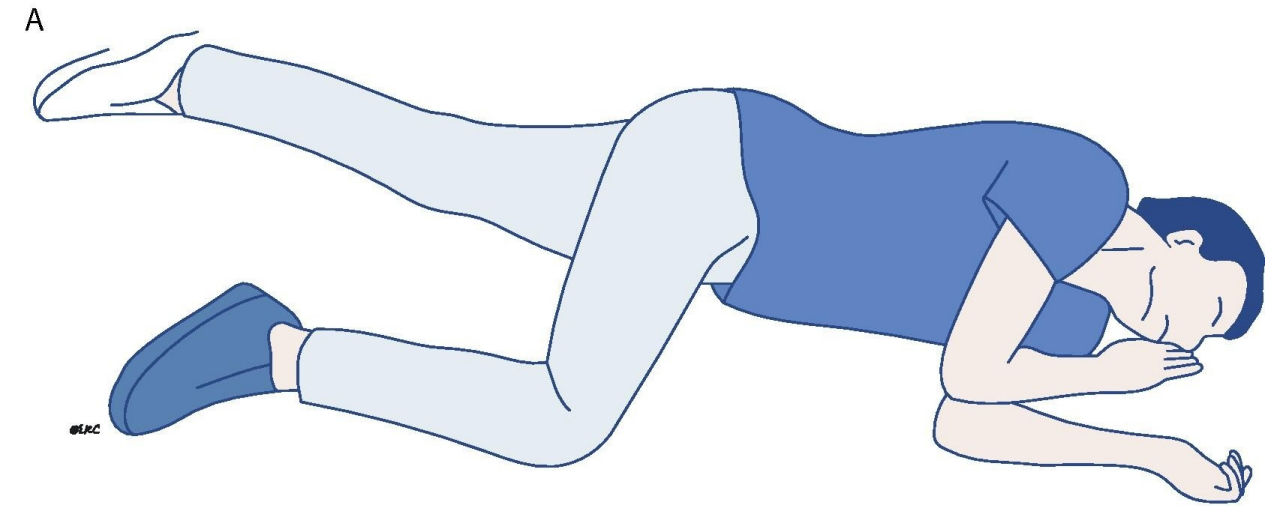
- ▶ Assess the collapsed or unwell person
- ▶ They do not respond but breathing normally
 - ▶ Put them in recovery position





RECOVERY POSITION

- ▶ Reassess for normal breathing every minute
- ▶ If breathing normally
 - ▶ Keep in the same position
 - ▶ Check every minute
- ▶ If not breathing normally
 - ▶ Roll onto back
 - ▶ Start basic life support



PRECAUTIONS WITH INFECTIONS

- ▶ Most people who do BLS [in the community] do it on someone they know
- ▶ The risk of transmission of disease causing serious harm to a rescuer is low
- ▶ Blood or vomit may increase the risk & also makes rescue breaths less appealing to many people



GENERAL RISK ASSESSMENT

- ▶ Individualised risk assessment

- ▶ Am I willing to perform BLS on this person?
 - ▶ Many rescuers know the person they are trying to resuscitate
 - ▶ Personal consideration of moral injury



Compression only CPR if you do
not give rescue breaths



CHILDREN



Cardiac arrest is rare

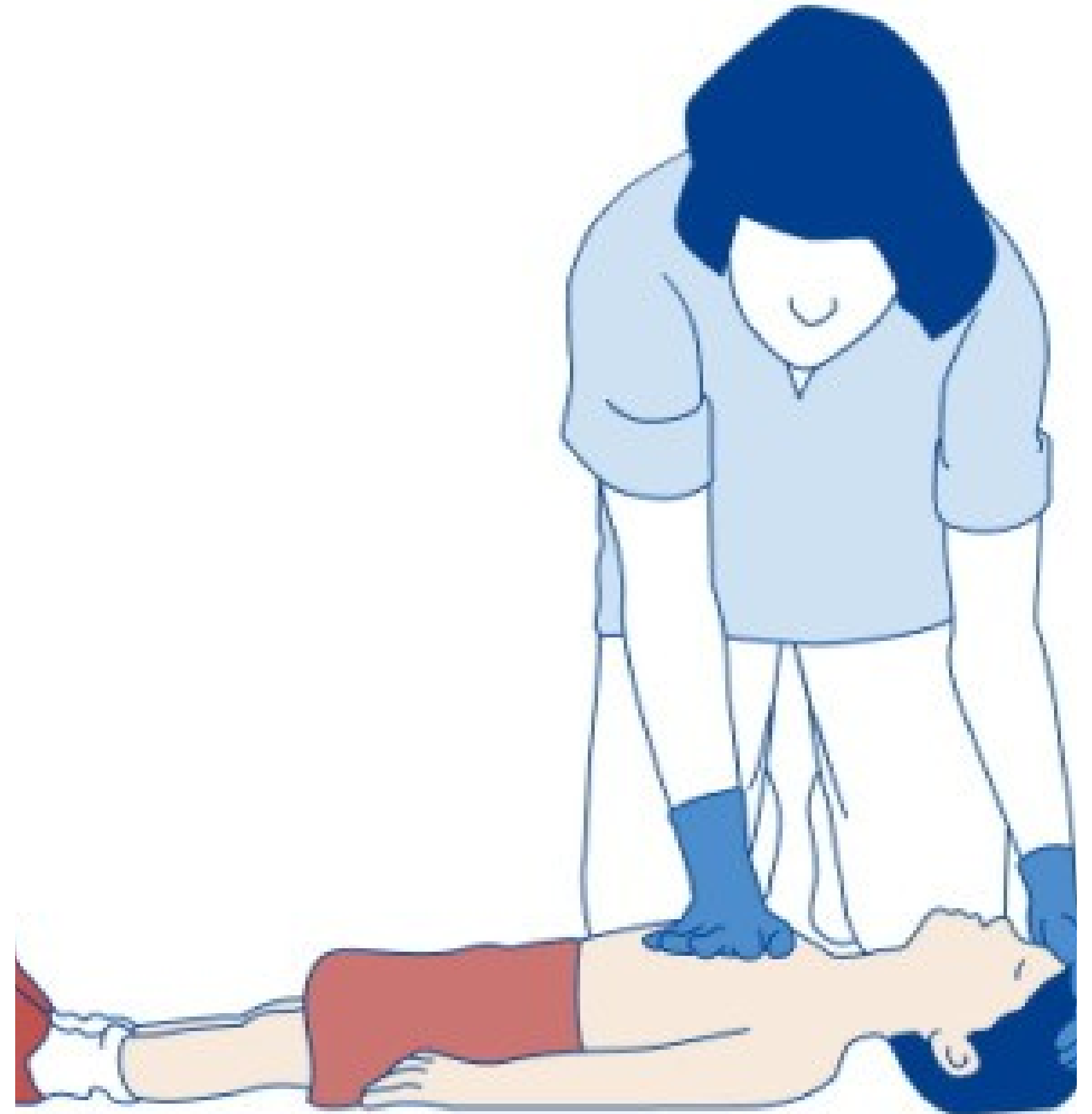
CHILDREN

- ▶ Modifications to improve the sequence
 - ▶ 5 initial rescue breaths before CPR
 - ▶ BLS for 1 minute before 999 call
- ▶ Compress 1/3 depth of the chest
 - ▶ Infants: 2 fingers
 - ▶ Small child: 1 hand
 - ▶ Large child: 2 hands



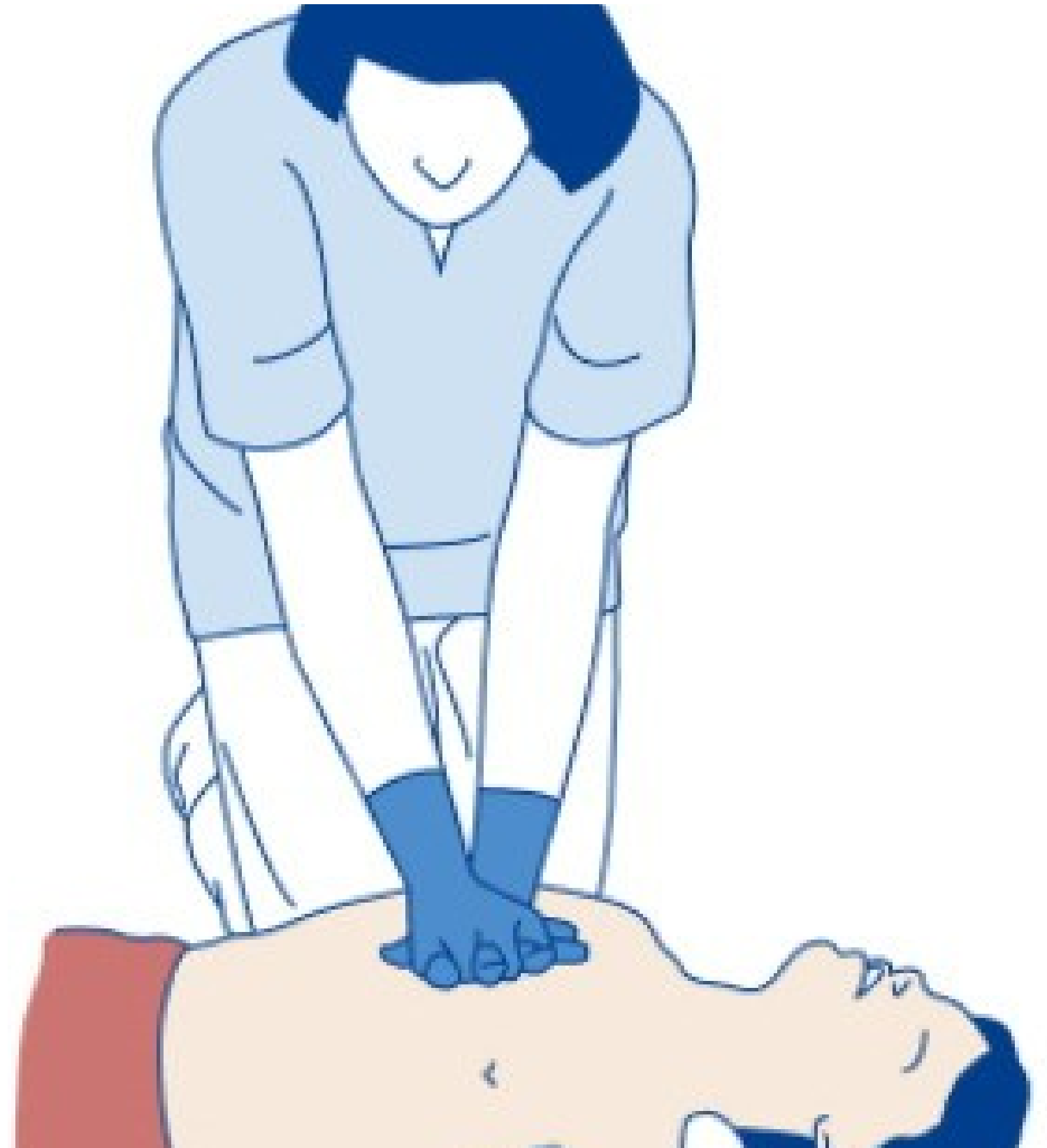
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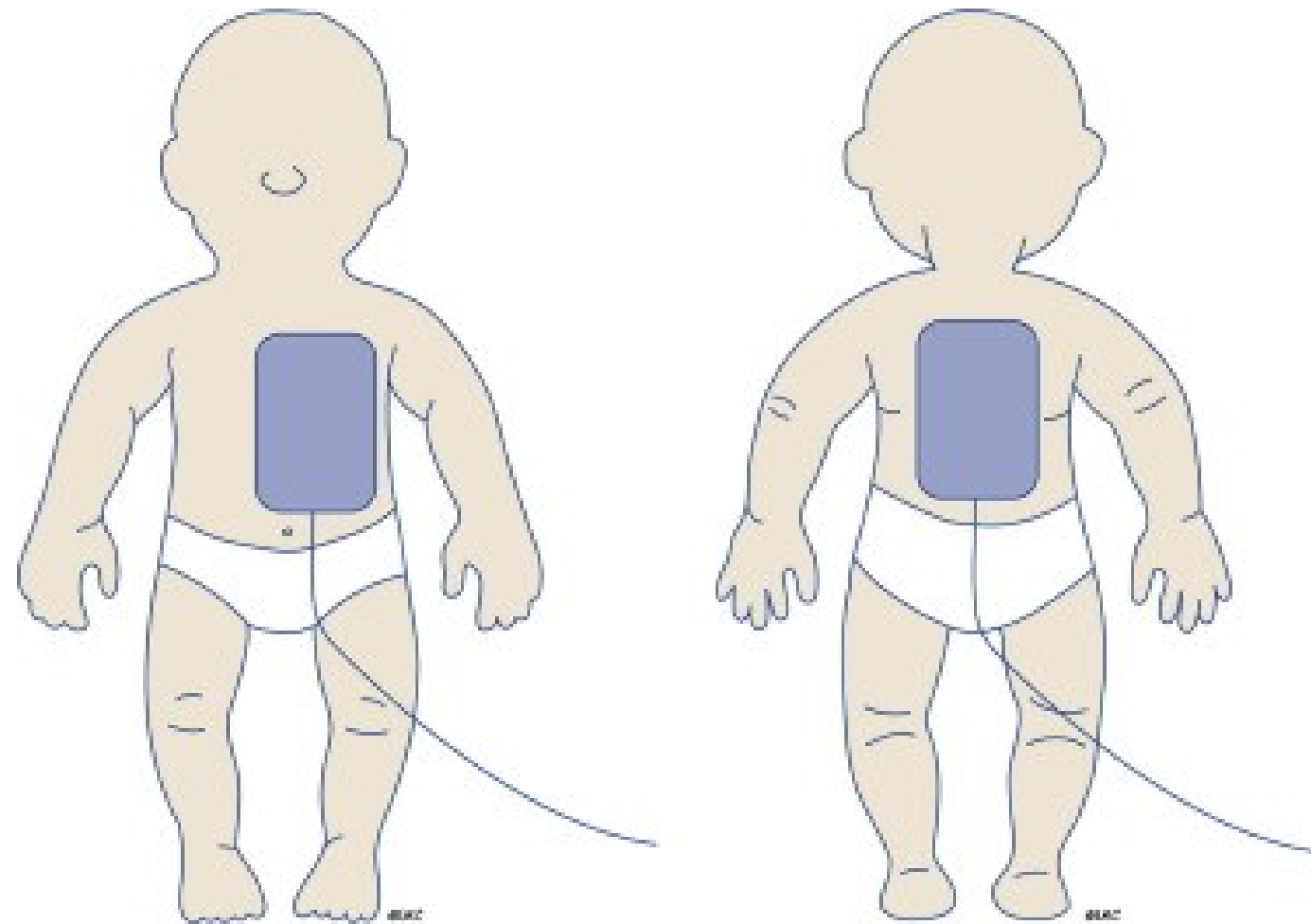


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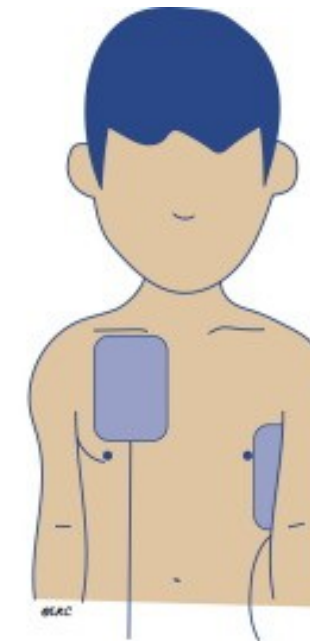
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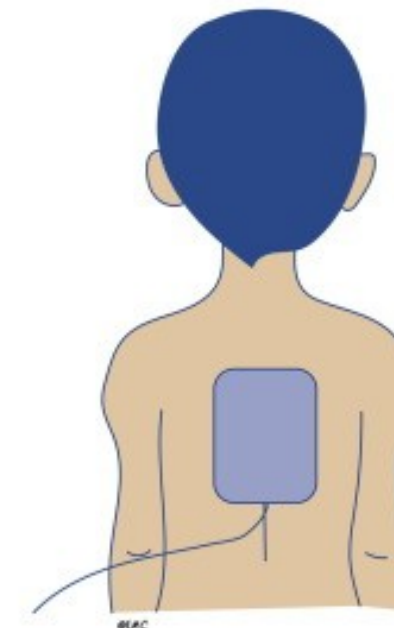
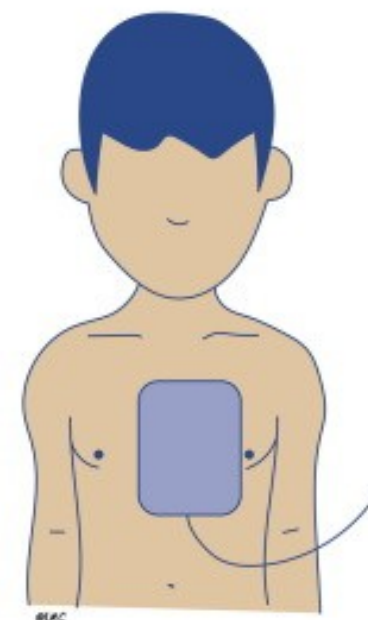
CHILDREN



a



b



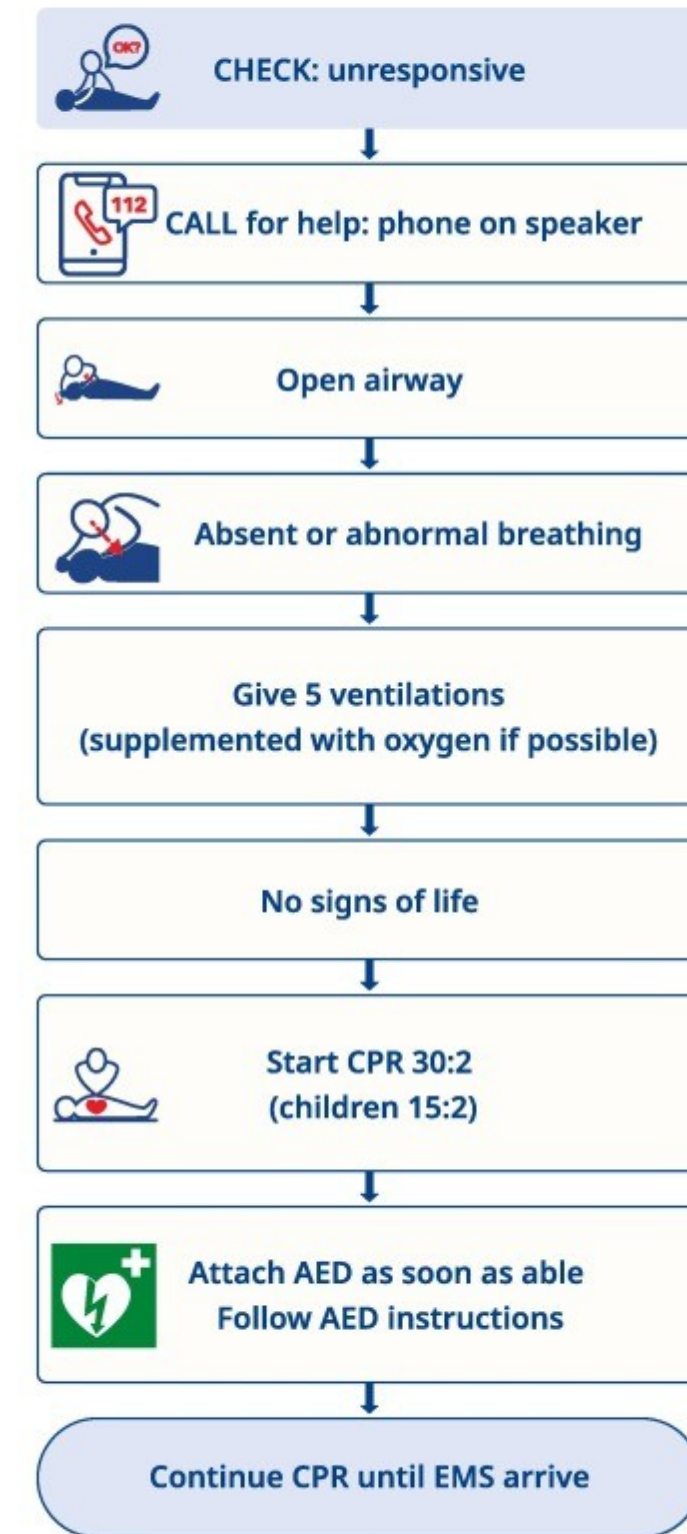
DROWNING

- ▶ Modifications to improve the standard sequence
 - ▶ 5 initial rescue breaths before CPR
 - ▶ BLS for 1 minute before 999 call



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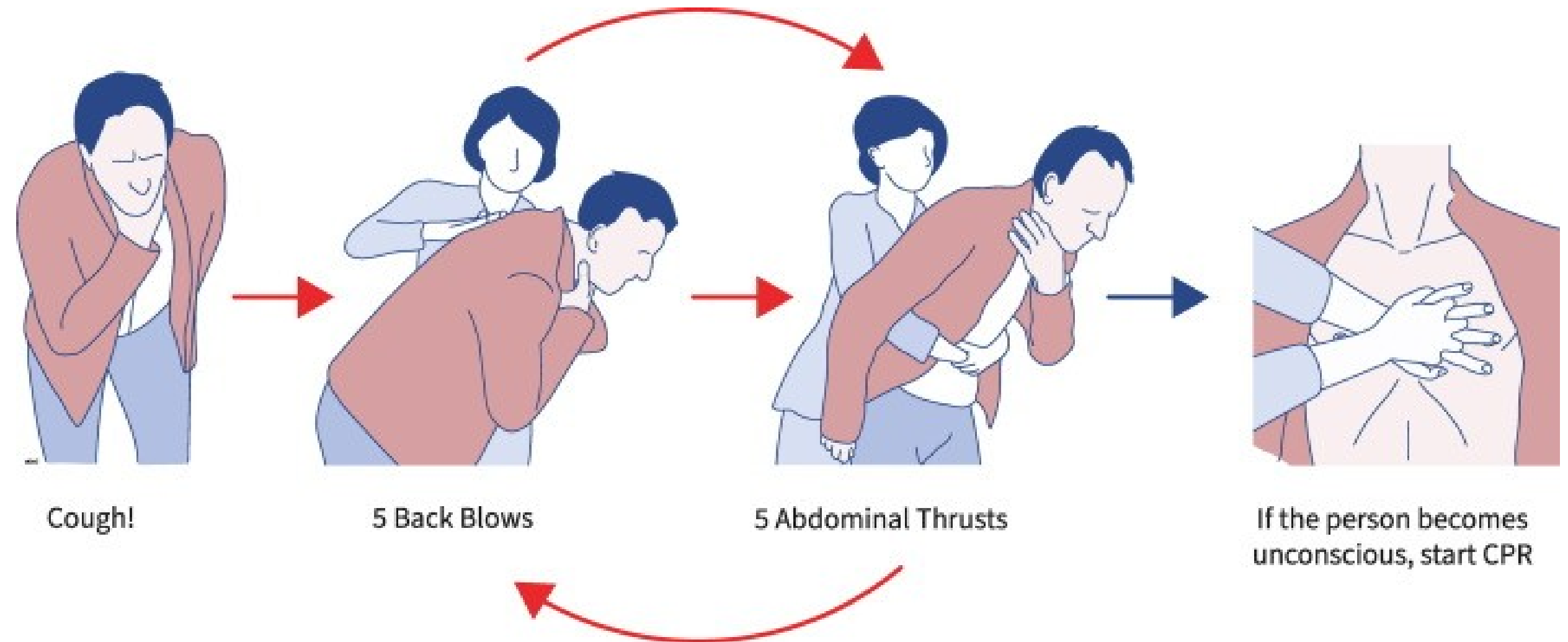
CHOKING



Most choking events associated with eating

CHOKING

- ▶ Suspect and recognise possibility of choking
- ▶ Encourage to cough



CHOKING

- ▶ Suspect and recognise possibility of choking
- ▶ Encourage to cough
- ▶ Can speak, cough and breathe
 - ▶ Mild airway obstruction



CHOKING

- ▶ Severe airway obstruction & conscious
 - ▶ 5 back blows



CHOKING

- ▶ Severe airway obstruction & conscious
 - ▶ 5 back blows
 - ▶ 5 abdominal thrusts



CHOKING

- ▶ Person becomes unconscious
- ▶ Support them to the ground
- ▶ 999
- ▶ Chest compressions



BLEEDING

- ▶ Most bleeding is minor
 - ▶ Wash the wound
 - ▶ Apply direct pressure
 - ▶ Localised cold therapy may be helpful



BLEEDING

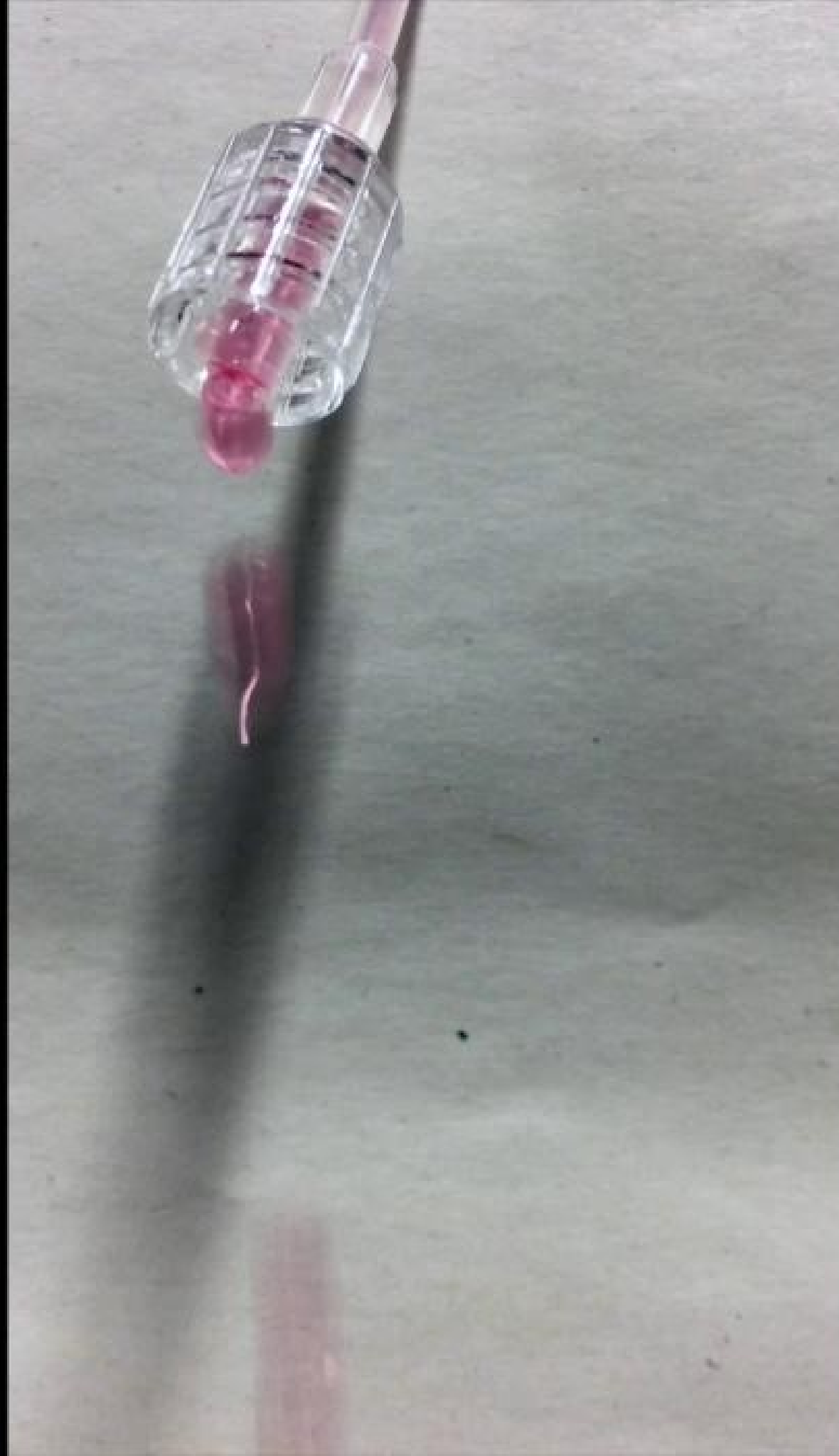
- ▶ Some bleeding is more serious
 - ▶ Apply direct pressure $\pm$ dressing



BLEEDING

- ▶ Some bleeding is catastrophic
 - ▶ Treat it first
 - ▶ Apply direct pressure $\pm$ dressing





BLEEDING

- ▶ Some bleeding is catastrophic
 - ▶ Apply direct pressure $\pm$ dressing
 - ▶ Tourniquet



BLEEDING

- ▶ Some bleeding is catastrophic
 - ▶ Stay safe
 - ▶ Act fast & take control
- ▶ Apply direct pressure $\pm$ dressing
- ▶ Tourniquet





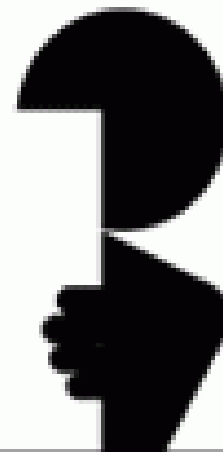
Manchester synagogue attack

B B C NEWS



**IN THE RARE EVENT OF
a firearms or weapons attack**

RUN HIDE TELL



ANY QUESTIONS?

SUMMARY

- ▶ Standard basic life support & automated external defibrillator algorithm
 - ▶ If in doubt, use this
- ▶ Minor modifications make it better in some circumstances
 - ▶ Paediatric resuscitation: 5 breaths, 1 minute before 999, $\frac{1}{3}$ depth of chest
 - ▶ Drowning: 5 breaths, 1 minute before 999
- ▶ Severe airway obstruction due to choking needs immediate treatment
 - ▶ Back slaps, abdominal thrusts, CPR
- ▶ Serious bleeding may need to be the first problem you treat
 - ▶ Direct pressure $\pm$ dressing, tourniquet

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